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Name: نادية علي مصطفى عابدين National ID: 26108282101185	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف روستاف
مستشفى
9-20

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

09/20/2026

MDT FORM

Name: نادية على مصطفى عابدين	MRN: 12627
Age: 65 Y	ID: 26108282101185
Menopausal Sta: post	Phone:* 01098666017
NRS SCORE:	Marital Status: married
BMI:	Family History: -ve
Gender:	Reason Of MDT: NEW
CO Morbidity: DM	ECOG Performance Status: 0 0 1 2 3 4
Surgical History:* cholecystectomy	

Clinical Data:

BX: RT IDC 2

ER: 8/8 **PR:** 6/8 **HER2:** amplified **KI67:** 15%

History:**Sonomammography:**

CESM RT multiple UOQ enhancing masses + PLN

Biomarker/Molecular

Luminal B2

Mets Workup:

ct cap free

bone scan free

Recommendations:**MDT Outcome & Plan**

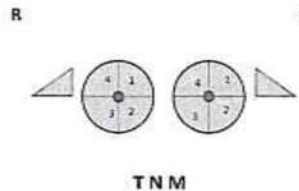
PORTA-CTH

ECHO

NA TCHPX6

MULTIFOCAL BUT LARGE BREAST FOR CLIPPING

RT WLE+ALND



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

T3 m N1 M0 + nipple retraction

Pathological Staging:**SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كيلاسي	د. محمد خير اوي		

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ناديه
على مصطفى عابدين
١٢ ش عبید الشرقاوى - المعصرة المحطة
المعصرة - القاهرة

١٩٦١/٠٨/١٨ ٢٦١٠٨٢٨٢١٠١١٨٥

١04435358



Patient Name	ناديه على مصطفى	Case No.	339300
Sex/Age	Female 64 Y	Patient No.	24301
Registered Date	29/6/2026 03:27:31 PM	Referred By Dr.	Interventional Radiology
Validation Date	20/7/2026 02:01:41 PM	Branch	Gustave Roussy
Reporting Date	25/7/2026 09:19:16 AM	Pathology No.	2061/26

Clinical Data: Right breast mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 29/06/2026, at 1:36 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Present.
- **Perineural invasion:** Absent
- **Tumor infiltrating lymphocytes (TILs):** Mild.
- **Necrosis:** Focal.

Results:

* **IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 40% of tumor cells (score 6/8).



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- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified. (Average Her2 copy No.: (4.95), Her2/CEP17 ratio: (2.1)).
- Ki67%: 15%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I. (Average Her2 copy No.: (4.95), Her2/CEP17 ratio: (2.1)).
- Ki67%: 15%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of



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the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name: Nadia Ali Mustafa
Date: 2026-06-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 64-year-old female with right breast palpable lump

Findings:

right breast.

- Dense fibroglandular densities. (ACR c) may obscuring small lesion
- center of outer third shows two ill defined high density mass lesions with groups of pleomorphic calcifications seen in between lesion
- **Ultrasound** demonstrated between 8-9 clock along duct shows multiple irregular heterogeneous hypo echoic solid mass lesions
 - index lesion at 8 o'clock, zone c 10cm from nipple, 11 mm deep to the skin measuring 20x23mm
 - inner to index lesion at 8 o'clock, way from it by 8mm 9cm from nipple, 16 mm deep to the skin measuring 16x12mm
 - at 9 o'clock, zone b 3cm from nipple, 12mm deep to the skin measuring 25x15mm
 - para areolar at 9 o'clock, 1.5 cm from nipple, 10 mm deep to the skin measuring 21x13mm
 - with prominent ducts in between lesions and reaching to nipple causing nipple retractions
 - two lymph node are seen at level I. largest measuring 18x8mm with focal cortical thickening 5mm and smallest measures 11x7mm and focal cortical thickening 3mm ??indeterminate

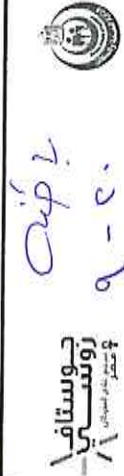
left breast.

- Dense fibroglandular densities. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion: -

- right breast (BIRADS 4c): multiple Malignant looking lesions for biopsy and CESM
- left breast (BIRADS 1): Normal study.

Best Regards,
DR. Marwa M.Hassan



مبادرة رئيس الجمهورية
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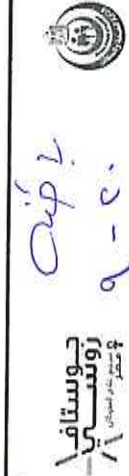
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كبراني	د محمد تاراري		



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د محمد كبراني	د محمد تاراري		